DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 71–80.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 71: The desk assignment

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Rogelio, 62, a fire captain with 34 years on the job, at an occupational health clinic on his department's referral. His wife, Norma, joins for part of the visit.

> *"The department says I have to talk to someone before they'll even think about putting me back on the engine. I'm fine. I just want my job back."*

If asked about…

When did this start?	Within a few days of the fire, four months ago. I figured it would pass. A few weeks later I froze at the door of another fire, and three months ago they put me on a desk.
What happened at the fire?	We were searching the second floor. The floor went, and I ended up pinned under a beam with my air alarm ringing. I thought, this is it. My crew cut me out. Bruised ribs, that's all.
Memories and dreams	Anything burning, even Norma's toast, puts me right back under that beam. Most nights I dream about it and wake up soaked.
What you avoid	I won't light the fireplace. I take the long way so I don't pass that street. I don't talk about it, not even with my crew.
How you see yourself	A captain should read a floor. I've told rookies that for twenty years. I let my guys down. With Norma and the grandkids I feel numb, like I'm watching them through glass.
Sleep and startle	Four hours, broken. When the tones go off on the radio at the desk, I jump. I sit where I can see the door.
Alcohol, drugs, medications	I don't drink. Never was much for it. Just my blood pressure pill, same one for years. Nothing new.
Medical history	Department physical last month, all normal. The ribs healed.
Thoughts of suicide or harming others	No. I think about how close I came, but I don't want to die. I have grandkids. I've never hurt anybody and I won't.
Work and family	I used to be the one people called. Now I push paper and snap at everybody. I quit going to the Friday cookouts at the station.
Family history	My father was in Korea. He never talked about it and he didn't sleep much. That's all I know.
Culture and context	In the firehouse you don't complain; you go back in. Norma wants me to talk to our deacon too. I might.

Only if asked (don’t volunteer)

- Was pinned under a beam four months ago and believed he would die
- Symptoms began within days of the fire, before the desk assignment
- Blames himself for not reading the floor
- Avoids the fireplace and the street where the fire was

Patient 71: answer and debrief

DIAGNOSIS

Posttraumatic stress disorder

Clues 1 and 2 point to an adjustment reaction to losing his place on the engine. Clue 3 flips it: being pinned in a fire and believing he would die is threatened death, a qualifying (Criterion A) event, which adjustment disorder does not require. Clues 4 to 6 show all four clusters: intrusion (smell-triggered memories, nightmares), avoidance (the fireplace, the street), negative thoughts and mood (self-blame, numbness) and arousal (irritability, poor sleep, startle), starting within days and lasting four months, more than the 1 month PTSD needs. Exam lesson: the separating detail is the event, not how upset the person is; with a Criterion A event and full criteria, diagnose PTSD, because adjustment disorder is the residual call. PTSD raises suicide risk, so ask directly; he denies any thoughts of suicide.

What had to be ruled out

- Adjustment disorder: a Criterion A event plus full PTSD criteria; adjustment disorder is used only when another disorder's criteria are not met
- Acute stress disorder: symptoms have lasted four months; that diagnosis covers only 3 days to 1 month after the event
- Major depressive disorder: the numbness and self-blame are tied to the fire; no pervasive low mood or thoughts of suicide
- Substance or medical cause: does not drink; only a long-standing blood pressure pill; normal department physical last month

Checkpoint: What would you ask or check NEXT?

A. Ask what led to the move off the engine, including any call where he was hurt, trapped or thought he might die **(best)**

B. Help him grieve the loss of his place on the crew and adjust to the desk role

C. Refer him to the department's peer support group for firefighters on light duty

A role change is an obvious stressor, but the event behind it decides between an adjustment reaction and PTSD, and first responders often face life-threatening events, so ask what happened first. B treats the role loss as the cause before anyone has asked. C may help later, but it skips the assessment that decides the diagnosis.

The case in six clues (as the app reveals it)

1. I'm short with everybody, I sleep maybe four hours a night, and I've stopped going to the Friday cookouts at the station.
2. Norma: Three months ago they took him off the engine and put him behind a desk. He's been with that crew since he was 28, and he hasn't been himself.
3. Four months ago a floor gave way under me at a house fire. I was pinned under a beam with my air alarm ringing, and I was sure I was going to die there. A few weeks later I froze at the door of another fire, and that's why they moved me.
4. Now the smell of anything burning puts me right back under that beam, and most nights I wake up from dreams about it. I won't light our fireplace, and I take the long way home so I don't pass that street.
5. I keep telling myself a captain should have read that floor. I feel numb with Norma and the grandkids, and when the tones go off on the station radio, I nearly jump out of my chair.
6. Norma: It started within days of the fire, before the desk job, and it hasn't let up in four months. He doesn't drink, he takes only his blood pressure pill, and his department physical last month was normal.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Generalized anxiety disorder
2. Adjustment disorder, Major depressive disorder, No mental disorder (normal or expected reaction)
3. Adjustment disorder, Posttraumatic stress disorder, Major depressive disorder
4. Posttraumatic stress disorder, Adjustment disorder
5. Posttraumatic stress disorder, Major depressive disorder
6. Posttraumatic stress disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Adjustment Disorders

Patient 72: The changed locks

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Zara, 24, a nursing assistant at a nursing home, at a housing program office. Her case manager, Monique, sits in with her permission.

> *"Monique said I should talk to you. I know it's just an apartment. I don't know why I can't get it together."*

If asked about…

When did this start?	The dreams started the first week after they locked me out. That was six weeks ago. Before that I slept fine.
What happened?	The building was sold and the new owner wanted everyone out. I fought it and lost in court. A deputy came at eight in the morning, and a locksmith changed the locks while I watched. The neighbors saw everything. Nobody yelled. It was just so humiliating.
Were you ever in danger?	No. Nobody touched me or threatened me. The deputy helped me carry my plants. I wasn't scared for my life. I was ashamed.
Dreams and memories	Same dream almost every night: me in the hall, the drill in the lock. I wake up with my heart pounding. A knock on any door makes me jump.
What you avoid	I go blocks out of my way so I don't see the building. The court letters are in a bag, unopened. I missed two showings because I couldn't make myself go in.
Mood	I'm not down all day. I eat, I laugh with my cousin, we watch our shows. It comes in waves, mostly with a knock or a letter. I don't feel worthless. I feel embarrassed.
Sleep	It takes me a while to fall asleep because I'm dreading the dream. Maybe five or six hours.
Thoughts of suicide or harming others	No. I've never thought about dying, and I'd never hurt anyone. I just want my own door again.
Alcohol, drugs, medications	I don't drink or smoke anything. Nothing but an allergy spray.
Medical history	Healthy. I had a checkup for work last month, and it was normal.
Work and past experiences	I've lost three shifts. I can't concentrate, and I was late twice after bad nights. Nothing like this ever happened to me before, and nobody ever hurt me growing up.
Family and culture	My mom says eviction happens to good people and there's no shame in it. In our family we help each other; that's why I'm on my cousin's couch. I still feel like I failed.

Only if asked (don’t volunteer)

- No one hurt or threatened her during the eviction; she was never in danger
- Mood comes in waves; appetite, enjoyment and self-worth intact
- Missed two apartment showings and lost three shifts
- Normal checkup last month; no substances

Patient 72: answer and debrief

DIAGNOSIS

Adjustment disorder

Nightmares, startle and avoidance after the eviction make PTSD a strong early guess. Clue 4 flips it: no one hurt or threatened her and she was never in danger, so the event does not meet PTSD Criterion A, however painful it was. Clue 5 shows too few symptoms for major depression. Onset within 3 months of the eviction, with missed showings and lost shifts, makes this adjustment disorder rather than a normal reaction. Exam lesson: PTSD and acute stress disorder require a Criterion A event; nightmares and avoidance after eviction, divorce or job loss point to adjustment disorder. Her housing problem can also be recorded with a Z code.

What had to be ruled out

- Posttraumatic stress disorder: the eviction did not involve actual or threatened death, serious injury or sexual violence (Criterion A)
- Acute stress disorder: same exposure requirement, and six weeks is past its 1-month window
- Major depressive disorder: appetite, enjoyment and self-worth intact; no thoughts of death
- No mental disorder (normal or expected reaction): missed showings and lost shifts show real impairment
- Substance or medical cause: no alcohol or drug use; normal checkup last month

Checkpoint: What would you ask or check NEXT?

A. Begin trauma-focused therapy for her nightmares and avoidance

B. Ask exactly what happened during the eviction, including whether anyone hurt or threatened her or she feared for her life **(best)**

C. Reassure her that bad dreams after a move are normal and should fade once she has a new place

Nightmares and avoidance fit PTSD only if the event involved actual or threatened death, serious injury or sexual violence, so ask what happened before deciding. A treats PTSD before that requirement is checked. C assumes a normal reaction without assessing her distress and impairment, which may amount to a disorder either way.

The case in six clues (as the app reveals it)

1. I have the same bad dream almost every night, and I jump every time someone knocks on a door. I can't think straight at work.
2. Monique: Zara was evicted six weeks ago from the apartment she'd rented for three years. She's sleeping on her cousin's couch while we look for a place.
3. In the dream I'm standing in the hall watching them change the locks. I go blocks out of my way so I won't pass my old building, and I haven't opened the letters from the court.
4. Nobody hurt me or threatened me that morning. The deputy was polite and even helped me carry my plants down. It was the most embarrassing day of my life, but I was never in danger.
5. I'm not down all the time. I still eat, I still laugh with my cousin, and I don't feel like a bad person or think about dying; it hits me when there's a knock or a letter comes.
6. Monique: She's missed two apartment showings and lost three shifts because she couldn't make herself go. She'd never seen a counselor before, she doesn't drink or use anything, and her checkup last month was normal.

What a skilled clinician weighs after each clue

1. Posttraumatic stress disorder, Generalized anxiety disorder, Nightmare disorder
2. Adjustment disorder, Posttraumatic stress disorder, Major depressive disorder
3. Posttraumatic stress disorder, Adjustment disorder
4. Adjustment disorder, Major depressive disorder
5. Adjustment disorder, No mental disorder (normal or expected reaction)
6. Adjustment disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Adjustment Disorders; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Posttraumatic Stress Disorder (Criterion A)

Patient 73: The Sunday cricket

Setting: Vikram, 52, a high school chemistry teacher, at a behavioral health visit in his primary care clinic. His daughter, Meera, came with him.

> *"Meera made the appointment. My wife died. Of course I'm sad. I don't see what talking will change."*

If asked about…

When did this start?	She died nine weeks ago. The first few weeks I was busy with the rites and the paperwork, and sad in the normal way. About six weeks ago it turned into this heaviness that never lifts.
Mood	It's there every minute, not just when I see her photo. Meera told me she got the job she wanted, and I felt nothing.
Grief and memories of her	I miss her, but honestly I think more about how useless I am than about her. Looking at our photos doesn't change anything; the weight is the same.
Self-worth	I've always been a failure. She covered for it. Now everyone can see it. I'm a burden on Meera.
Interest and activities	I quit cricket. My grandson visits and I can't make myself play with him. Nothing feels like anything.
Sleep, appetite, energy and concentration	I wake at four every morning. Food tastes like paper, and I've lost weight without trying. I read the same email five times. I've been on leave from work for a month.
Thoughts of suicide	Some nights I wish I wouldn't wake up. I have no plan, and I would never do that to Meera. There are no guns in the house, and the hospice nurse took back my wife's medicines.
Past highs	Never. I've always been steady, maybe too steady.
Alcohol, drugs, medications	I don't drink. I take nothing, not even for blood pressure.
Medical history	My doctor did blood tests last week, thyroid too. All normal.
Family history	My father went very quiet for a year after his business failed. Back then no one called it anything.
Culture and context	We did the thirteen days of rites and the prayers, and our temple community has been kind. They tell me to be strong for the family. I'm trying, and it isn't working.

Only if asked (don’t volunteer)

- Low mood is constant, not tied to reminders of his wife
- Feels worthless and a burden
- Passive wish not to wake up; no plan or intent; no guns, and his wife's medicines were returned
- On leave from work for a month

Patient 73: answer and debrief

DIAGNOSIS

Major depressive disorder

Two months after his wife's death, the first guess is normal grief, and prolonged grief disorder cannot apply before 12 months in an adult. Clue 5 flips it: the low mood is constant rather than coming in waves tied to reminders, and he feels worthless and a burden, not just bereft. With early waking, poor appetite, loss of interest in everything, poor concentration, thoughts of death and six weeks of daily symptoms, he meets major depressive disorder. Exam lesson: DSM-5 removed the bereavement exclusion, so diagnose MDD during grief when full criteria are met; grief keeps self-esteem intact, depression attacks it. His passive wish not to wake up, with no plan or intent, still calls for a safety plan, a check on means at home, involving Meera with his consent and close follow-up.

What had to be ruled out

- Prolonged grief disorder: only nine weeks since the death; adults need 12 months
- No mental disorder (normal or expected reaction): constant low mood, worthlessness and loss of interest in everything go beyond grief, and he cannot work
- Adjustment disorder: full MDD criteria are met, and adjustment disorder is the residual call
- Bipolar II disorder: no past highs
- Depressive disorder due to another medical condition or a substance: normal blood tests and thyroid; no alcohol or medications

Checkpoint: What would you ask or check NEXT?

A. Reassure him that guilt and sadness are part of normal grief and plan to check in again in a few months

B. Refer him to a weekly support group for people who have lost a spouse

C. Ask directly about thoughts of suicide, including any plan, intent, means at home and what keeps him safe **(best)**

Calling himself a burden is a suicide warning sign, so assess risk directly and now. A misses the warning sign and wrongly files worthlessness under normal grief. B may help later, but it does not address current risk or the depression.

The case in six clues (as the app reveals it)

1. I've always been a steady person, no big highs. Now I wake at four and can't get going, and food has no taste, so I hardly eat.
2. Meera: My mother died of cancer. Dad cared for her at home through all of it.
3. It's been nine weeks since the funeral. The hospice nurse told us the first months are the hardest and to give it time.
4. Meera: He used to light up around his grandson. Now he sits through the visits without a smile, and he's quit the Sunday cricket he's played for thirty years.
5. It isn't only when I think of her. The heaviness is there every minute, even when there's good news, and I've been on leave from work for a month because I can't concentrate. I've always been a failure, and now I'm just a burden on everyone.
6. Some nights I wish I wouldn't wake up, but I have no plan, and I would never do that to Meera. It has been like this every day for about six weeks. I don't drink, I take no medicines, and my blood tests last week, thyroid too, were normal.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Depressive disorder due to another medical condition
2. No mental disorder (normal or expected reaction), Prolonged grief disorder, Major depressive disorder
3. No mental disorder (normal or expected reaction), Major depressive disorder, Adjustment disorder
4. Major depressive disorder, No mental disorder (normal or expected reaction)
5. Major depressive disorder, Adjustment disorder
6. Major depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder; Stanley & Brown (2012), Cognitive and Behavioral Practice 19(2), Safety Planning Intervention: a brief intervention to mitigate suicide risk

Patient 74: Route 14

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Odessa, 54, a school bus driver on unpaid leave, at a hospital pain management clinic. Her husband, Marvin, came with her.

> *"Another doctor? Fine. But I'm telling you, something is wrong with my spine, and nobody is listening."*

If asked about…

When did this start?	About ten months ago. I lifted a little boy's wheelchair onto my bus, felt a pull, and it never went away.
The pain	Every waking hour. Most days a seven out of ten, worse when I sit. It shoots down my right leg. Some mornings Marvin has to help me sit up.
What you fear	That it's a tumor or something eating my spine, and they'll find it too late. Or that I'll end up in a wheelchair myself.
Tests and doctors	Two MRIs and four specialists. They all say disc wear that's normal for my age. I've asked for a third MRI.
Time and activities	I read about spine diseases on my phone for hours at night. I won't drive, lift groceries or pick up my granddaughter, because it could make it worse.
Sleep and mood	I sleep in pieces because of the pain. My mood is all right when the pain eases. I still laugh at our shows with Marvin.
Alcohol, drugs, medications	I don't drink. I take the anti-inflammatory my doctor prescribed, the way she says, and I do my stretches.
Medical history	High blood pressure, well controlled. Nothing else.
Thoughts of suicide	No. Never. I want my life back.
Work	I've driven route 14 for nineteen years. Those kids are my kids. I'm on unpaid leave and I want to go back, but I'm scared I'll hurt myself.
Family history	My mother had arthritis and worked through it. No one in my family has had mental health problems that I know of.
Culture and context	At church they pray over me every Sunday. My pastor says to trust the doctors. Marvin says I've stopped living.

Only if asked (don’t volunteer)

- The pain itself is severe and constant, not only the fear
- Scans show mild disc wear that is normal for her age
- Spends hours each night reading about spine diseases
- Pain began ten months ago; she is on unpaid leave and wants to return

Patient 74: answer and debrief

DIAGNOSIS

Somatic symptom disorder

Many doctors, scans and fear of a hidden tumor make illness anxiety disorder the early lead. Clue 4 flips it: the pain itself is severe, constant and disabling, so the body symptom, not only the fear, is at the center. Hours of reading, the belief that she will end up in a wheelchair, and giving up driving and lifting, all out of proportion to mild disc findings, are the excessive thoughts, feelings and behaviors, and they have lasted ten months, past the typical 6 months. This is somatic symptom disorder with predominant pain. Exam lesson: in somatic symptom disorder the symptoms are prominent and distressing; in illness anxiety disorder they are absent or mild. A real medical finding does not rule out SSD, because the excessive response is the focus. One coordinating doctor with regular scheduled visits, CBT and active rehab help.

What had to be ruled out

- Illness anxiety disorder: her pain is severe and prominent, not absent or mild
- Malingering or factitious disorder: no sign of feigning or deception; she is on unpaid leave and wants to return to work
- Major depressive disorder: mood lifts when the pain eases; no pervasive low mood
- Generalized anxiety disorder: worry centers on her back, not many areas of life
- Substance cause: no alcohol; takes only her prescribed medicine as directed

Checkpoint: What would you ask or check NEXT?

A. Refer her to a spine surgeon for another opinion to settle her fear

B. Tell her the scans were reassuring so she can stop worrying

C. Ask about the pain itself (how strong, how often, what it keeps her from doing) and, with her consent, get her doctors' findings **(best)**

Whether distress centers on a prominent, disabling symptom or on fear of disease with mild symptoms separates somatic symptom disorder from illness anxiety disorder, and the medical findings need to be known, so ask and coordinate. A adds more testing, which feeds the cycle when results so far explain little. B is dismissive, reassurance alone rarely lasts, and it skips the assessment.

The case in six clues (as the app reveals it)

1. I'm on leave from my job, I hardly sleep, and I've been to more doctors this year than in my whole life.
2. I've had two MRIs and seen four specialists, and nobody will tell me what's really wrong. I'm scared it's something serious they keep missing, like a tumor on my spine.
3. Marvin: She reads about spine diseases on her phone for hours every night. Every new article sends her back to the doctor.
4. It's not only worry. My lower back hurts every waking hour, a seven out of ten, and it shoots down my leg. Some mornings Marvin has to help me sit up.
5. The doctors say the scans show some disc wear that's normal for my age and can't explain pain this bad. I don't believe that. I won't drive or lift a grocery bag, because I'm sure I'll end up in a wheelchair.
6. Marvin: It started when she lifted a child's wheelchair onto the bus about ten months ago, and it's run our life ever since. She doesn't drink, she takes only the medicine her doctor prescribed, her mood is fine when the pain eases, and she wants her route back.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Somatic symptom disorder, Illness anxiety disorder
2. Illness anxiety disorder, Somatic symptom disorder, Generalized anxiety disorder
3. Illness anxiety disorder, Somatic symptom disorder
4. Somatic symptom disorder, Illness anxiety disorder
5. Somatic symptom disorder, Malingering
6. Somatic symptom disorder

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Somatic Symptom Disorder; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Illness Anxiety Disorder

Patient 75: The rest-stop videos

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Wesley, 46, a regional sales rep for a seed company who covers three states by car, in an employee assistance program telehealth session from his car at a rest stop. His wife, Rhonda, joins by phone from home.

> *"The EAP said I should do this for my wife. I'm fine. I just need to know there's nothing growing in my head, without anybody looking."*

If asked about…

When did this start?	Almost three years ago. My brother Nate died of a brain tumor at fifty. His started with headaches. A month after the funeral, I started watching mine.
What you feel	Maybe once a week, a dull ache across my forehead after a long day of driving. A nap fixes it. No blurry vision, no weakness, nothing else.
What you fear	That it's what Nate had, and by the time anybody finds it, it's too late.
Checking	Every night Rhonda watches me on video: finger to nose, heel to toe, reading a sign far away. I always pass. Then I read about tumors at the next rest stop anyway.
Doctors	I haven't seen a doctor in three years. Rhonda booked two checkups and I canceled both. If they find something, I don't want to hear it. Driving past a hospital makes my chest go tight.
Other worries	Money's tight like anybody's, but I don't lose sleep over it. It's just this.
Sleep and mood	I lie awake in motels thinking about it, maybe five hours a night. I'm not depressed. I still go fishing with my son when I'm home.
Alcohol, drugs, medications	A beer or two on weekends at home. No drugs. I won't even take a pain pill; I'd rather sleep the headache off.
Thoughts of suicide	No. I want to be around. That's the whole point.
Grief	I miss Nate. We tell stories about him at Thanksgiving and laugh. It's not that I can't let him go. It's that I think I'm next.
Family history	Nate's tumor, and my mom had high blood pressure. Nobody in the family has had anxiety that I know of, but we wouldn't say.
Work and family	My sales numbers are fine, but I lose hours at rest stops reading. Rhonda's worn out from the videos.

Only if asked (don’t volunteer)

- His only symptom is a mild weekly headache relieved by a nap
- Has canceled two checkups and has not seen a doctor in three years
- Began after his brother died of a brain tumor almost three years ago
- His nightly self-checks on video are always normal

Patient 75: answer and debrief

DIAGNOSIS

Illness anxiety disorder

His focus on his head and nightly checks first fit somatic symptom disorder. Clue 4 flips it: the only symptom is a mild weekly headache that a nap clears, and the distress is about what it might mean. Nearly three years of preoccupation, daily checking and avoiding doctors so he won't hear bad news make this illness anxiety disorder, care-avoidant type. Exam lesson: in illness anxiety disorder symptoms are absent or mild and the fear is the problem; care-avoidant clients stay away from doctors, while care-seeking clients go too often. Either way, connect him gently with one primary doctor for a baseline exam, and offer CBT.

What had to be ruled out

- Somatic symptom disorder: his only symptom is a mild weekly headache; the fear, not the symptom, drives the distress
- Generalized anxiety disorder: worry is limited to one illness
- Obsessive-compulsive disorder: the checks serve only his fear of a tumor; no other obsessions or rituals
- Panic disorder: no panic attacks
- Prolonged grief disorder: he misses his brother, but the preoccupation is with his own health, not yearning for the person who died
- A medical cause of the headaches: brief, mild and relieved by rest, with normal nightly checks; a baseline exam still belongs in the plan

Checkpoint: What would you ask or check NEXT?

A. Accept his choice to stay away from doctors, since his symptoms are so mild

B. Explore what keeps him from seeing a doctor and, with his consent, work with Rhonda and one primary care doctor toward a single, well-paced checkup **(best)**

C. Help him arrange an urgent brain scan so the question is settled once and for all

A care-avoidant client still needs a medical home: one primary doctor and a baseline exam check for disease and limit cycles of fear and reassurance, and exploring the avoidance opens the door to treatment. A leaves him without care and leaves the fear untreated. C jumps to urgent testing that his mild symptoms do not call for, before any doctor has examined him, and repeated testing tends to feed the fear.

The case in six clues (as the app reveals it)

1. I don't sleep well on the road, and my mind won't shut off. Rhonda says I haven't been myself in a long time.
2. It's my head. I'm afraid something is wrong in there, and I think about it most of the day while I drive.
3. Rhonda: Every night he has me watch him on video touch his nose and walk heel to toe, and he always passes. He reads about tumors at every rest stop. But he won't make a doctor's appointment.
4. To be honest, I barely feel anything. Maybe a dull headache once a week after a long day, gone after a nap. It's what it might be that I can't shake, because that's how my brother's tumor started.
5. Rhonda has booked me two checkups, and I canceled both. If there's something in my head, I don't want to hear it. Just driving past a hospital makes my chest go tight.
6. It started almost three years ago, right after my brother died of a brain tumor at fifty. I don't worry like this about money or the kids; it's only this. I've never had a panic attack, and I have a beer or two on weekends, nothing else.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Major depressive disorder, Insomnia disorder
2. Somatic symptom disorder, Illness anxiety disorder, Generalized anxiety disorder
3. Somatic symptom disorder, Illness anxiety disorder, Obsessive-compulsive disorder
4. Illness anxiety disorder, Somatic symptom disorder
5. Illness anxiety disorder, Generalized anxiety disorder
6. Illness anxiety disorder

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Illness Anxiety Disorder; DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Somatic Symptom Disorder

Patient 76: The library shelves

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Tova, 34, a public librarian, at a behavioral health visit in a community health center. Her roommate, Wren, came with her.

> *"I'm not crazy. Something is wrong with my heart, and the ER keeps sending me home."*

If asked about…

When did this start?	The first one was about three months ago, at my desk on an ordinary afternoon. Since then I've had maybe six, and two were bad enough for the ER.
What happens	My heart pounds, my chest gets tight, I can't get a breath, my hands tingle, and I'm sure I'm dying. It peaks in about ten minutes. Afterward I'm wiped out.
When they happen	Anywhere. Once it woke me at two in the morning. Once I was shelving books on a dead-quiet afternoon. Nothing sets them off that I can find.
Medical tests	Two ER visits with heart tracings and blood work, a two-day heart monitor, thyroid tests. All normal. The cardiologist said my heart is healthy.
Caffeine, alcohol, drugs, medications	One coffee in the morning. A glass of wine at book club maybe twice a month. No drugs, no energy drinks, no cold medicine. Just birth control pills for years.
Worry and changes	I think about the next one all day. I stopped running because a fast heartbeat scares me. I sleep with my phone in my hand. It's been about two months like this.
Places you avoid	I don't skip places. I ride the subway, I go to the market on Saturdays, I went to a packed concert with Wren. I'm nervous, but I go.
Sleep and mood	My sleep's lighter since the night one. I'm not depressed. I'm frustrated.
Thoughts of suicide	No. The whole problem is that I'm scared of dying.
Family history	My dad had 'nerves.' He'd get dizzy spells and have to sit down. Nobody ever named it.
Work and context	I've left work early twice. My director has been kind. I'm usually the one who stays calm when a patron gets loud, so this is embarrassing.

Only if asked (don’t volunteer)

- One attack woke her from sleep; no trigger can be found
- Normal heart tracings, heart monitor and thyroid tests
- Stopped running and sleeps holding her phone for two months
- Still rides the subway, shops in crowds and goes to concerts

Patient 76: answer and debrief

DIAGNOSIS

Panic disorder

Pounding heart, air hunger and two ER visits first raise a medical cause. Clue 4 flips it: normal heart tracings, a normal heart monitor and normal thyroid tests, with little caffeine and no stimulants, make a medical or substance cause unlikely. The attacks are unexpected (one woke her from sleep), and two months of worry about more attacks with changed behavior meets the 1-month threshold. Clue 6 rules out agoraphobia: she still rides the subway, shops in crowds and goes to concerts. Exam lesson: rule out heart, thyroid and stimulants first; panic disorder needs unexpected attacks plus at least 1 month of worry or behavior change; add agoraphobia only if she fears or avoids 2 or more situation types. CBT with interoceptive exposure and SSRIs are first-line.

What had to be ruled out

- Anxiety disorder due to another medical condition: normal heart tracings, heart monitor and thyroid tests
- Substance or medication cause: one coffee a day; no stimulants, energy drinks or decongestants
- Agoraphobia: no fear or avoidance of 2 or more situation types; she rides the subway, shops in crowds and goes to concerts
- Generalized anxiety disorder: worry is about having more attacks, not many life areas
- Illness anxiety disorder: her fear follows real attacks and centers on the next attack, not on a hidden disease

Checkpoint: What would you ask or check NEXT?

A. Ask what medical tests she has had, especially of her heart and thyroid, and about caffeine, stimulants, decongestants and other substances **(best)**

B. Teach slow breathing and start CBT for panic

C. Ask which places she now avoids because of the attacks

Chest pain, air hunger and fear of dying call for ruling out heart, thyroid and substance causes before naming a panic disorder. B treats a diagnosis that has not been made. C is a fair question later, to check for agoraphobia, but it skips the medical and substance rule-out that comes first.

The case in six clues (as the app reveals it)

1. Twice in the last three months I've ended up in the ER sure I was having a heart attack. I'm scared to be alone at night now.
2. It hits out of nowhere: my heart pounds, I can't get air, my hands tingle, and I feel like I'm about to die. It's at its worst within ten minutes, and afterward I'm wiped out.
3. Wren: She's had five or six of these, not just the two that sent her to the ER. One woke her up at two in the morning; another came while she was shelving books on a quiet afternoon. We can't find any pattern.
4. The ER did heart tracings and blood tests both times, and my doctor had me wear a heart monitor for two days and checked my thyroid. Everything was normal. I have one cup of coffee in the morning, no energy drinks, no cold pills, nothing else.
5. Now I worry all day about the next one. I stopped running and I sleep with my phone in my hand, and it's been like this for two months.
6. Wren: She still takes the subway to work every day, shops at the crowded market on Saturdays, and came to a packed concert with me last week. She's nervous about another attack, but she doesn't skip places.

What a skilled clinician weighs after each clue

1. Anxiety disorder due to another medical condition, Panic disorder, Illness anxiety disorder
2. Anxiety disorder due to another medical condition, Panic disorder
3. Panic disorder, Anxiety disorder due to another medical condition
4. Panic disorder, Agoraphobia, Generalized anxiety disorder
5. Panic disorder, Agoraphobia
6. Panic disorder

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Panic Disorder; DSM-5-TR (APA, 2022), Anxiety Disorders: Agoraphobia

Patient 77: The dropped bolts

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Omar, 44, a city bus mechanic and father of two, at a behavioral health visit in his primary care clinic. His wife, Samira, came with him.

> *"My doctor sent me down the hall. I think I'm just stressed, but Samira says I'm not myself."*

If asked about…

When did this start?	About three months ago. It came on fast. Before that I was the calm one in the garage.
Worry	Everything. Bills, the kids, whether I locked the shop. My mind won't slow down.
Heart and body	A few times my heart pounded so hard at rest I had to sit on the floor. I'm hot all the time, I sweat in a cold garage, my hands shake, and I've lost fifteen pounds while eating everything in sight.
Sleep and energy	I'm exhausted but I can't fall asleep. My mind and my heart are both racing. I don't feel full of energy. I feel worn out.
Mood	Not sad. On edge. I still love taking the kids to the park.
Caffeine, alcohol, drugs, medications	Two cups of tea a day, same as for twenty years. I don't drink alcohol. No drugs, no supplements, no workout powders. Just the two pills my doctor started.
Medical history	Healthy until this. My doctor said my thyroid is very overactive. She's sending me to a specialist.
Since starting treatment	Ten days on the medicines, and the shaking is less and my heart isn't pounding. I slept six hours last night.
Thoughts of suicide or harming others	No. Never. I snap at the kids and then I feel terrible, but I've never hit anyone.
Family history	My mother's sister had a thyroid problem. She had an operation on her neck.
Work and culture	My supervisor took me off brake jobs because of the shaking. I've always fasted for Ramadan without trouble, but with this weight loss my doctor wants me to talk to her first. Our imam says health comes first.

Only if asked (don’t volunteer)

- Never anxious before; onset about three months ago
- Weight loss despite a big appetite, heat intolerance and tremor
- His doctor found a very overactive thyroid and started treatment ten days ago
- Symptoms are easing on treatment

Patient 77: answer and debrief

DIAGNOSIS

Anxiety disorder due to another medical condition

Restlessness, worry and poor sleep suggest generalized anxiety disorder, and the pounding heart suggests panic. But the anxiety came on fast at 44 in a man who was never a worrier, a medical red flag, and clue 4 adds weight loss despite a big appetite, heat intolerance and tremor. Clue 5 confirms an overactive thyroid, and clue 6 shows the anxiety easing as it is treated. This is anxiety disorder due to another medical condition (hyperthyroidism). Exam lesson: new anxiety in midlife with physical signs calls for a medical exam first; GAD would also need 6 months. Untreated hyperthyroidism can strain the heart, so medical care comes before therapy.

What had to be ruled out

- Generalized anxiety disorder: began three months ago along with the thyroid problem; GAD needs 6 months and no better medical explanation
- Panic disorder: the pounding-heart spells are explained by the overactive thyroid
- Substance or medication cause: same tea for years; no supplements, other medications or drugs
- Bipolar disorder: racing thoughts without elevated mood or grandiosity; he is exhausted, not full of energy
- Major depressive disorder: his mood is anxious, not low, and he still enjoys his children

Checkpoint: What would you ask or check NEXT?

A. Teach relaxation skills and start CBT for worry

B. With his consent, check with his doctor about a prompt exam and blood tests, including thyroid tests **(best)**

C. Refer him to psychiatry for an anti-anxiety medication

New anxiety at 44 with weight loss despite a big appetite, heat intolerance and tremor points to a medical cause, most likely the thyroid, so a medical workup comes first; he is already in his primary care clinic, so coordinate with his doctor. A treats a psychiatric diagnosis before the medical rule-out. C adds a medication without first finding the cause.

The case in six clues (as the app reveals it)

1. I'm on edge all the time. I can't sit still, I snap at the kids, and I lie awake at night.
2. I worry about everything: the bills, the kids, whether I locked the shop. My mind races and I can't turn it off.
3. Samira: A few times his heart has pounded so hard while he was resting that he had to sit down on the garage floor. And this all came on fast, over about three months. He's never been a worrier.
4. I eat like a teenager and I've still lost fifteen pounds. I'm sweating in the garage when everyone else wears a jacket, and my hands shake so much I drop bolts.
5. Samira: His doctor ran blood tests last week. She said his thyroid is very overactive and started him on a medicine for it and another to slow his heart.
6. That was ten days ago, and the shaking and pounding are already easing. I drink the same two cups of tea I always have, I take no supplements or other pills, and I've never used drugs.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Major depressive disorder, Insomnia disorder
2. Generalized anxiety disorder, Major depressive disorder
3. Anxiety disorder due to another medical condition, Panic disorder, Generalized anxiety disorder
4. Anxiety disorder due to another medical condition, Generalized anxiety disorder
5. Anxiety disorder due to another medical condition, Generalized anxiety disorder
6. Anxiety disorder due to another medical condition

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Anxiety Disorder Due to Another Medical Condition; Ross et al. (2016), Thyroid 26(10), American Thyroid Association guidelines for the diagnosis and management of hyperthyroidism and other causes of thyrotoxicosis

Patient 78: The window seat

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Nell, 52, a hospital quality manager just promoted to a regional role, at an outpatient telehealth visit. Her wife, Harriet, joins for part of the visit.

> *"I have to get on a plane in five weeks, and I can't. I need you to fix this fast."*

If asked about…

When did this start?	Fifteen years ago. A flight hit bad turbulence for a few minutes. Drinks flew and people screamed. Nobody was hurt, and the pilot said it was routine, but I haven't flown since.
What you fear	That the plane will drop and I'll be trapped in that tube with no way off. Even planning a trip makes my heart race.
Panic attacks	Only on a plane, which I don't do anymore, or when I'm booking one, like last week at the kitchen table. Never out of the blue.
Other places	Buses, trains, elevators, crowds, big stores, hiking alone in open country: all fine. I take the city bus every day. Harriet and I go to stadium games.
Memories and dreams	I don't have nightmares or flashbacks about that flight. I just won't fly.
Sleep and mood	Sleep's fine except the nights after I look at flights. My mood is good. I'm excited about the job, apart from this.
Alcohol, drugs, medications	A glass of wine with dinner. No drugs. Only a cholesterol pill.
Medical history	My checkup two months ago was normal. No heart problems.
Thoughts of suicide	No, never.
Work	The regional role means flying to six hospitals, starting with a training in five weeks. I canceled the first trip. If I can't fly, I'll have to turn the promotion down.
Family and culture	My mom never flew either; she called planes tin cans. Harriet grew up in a military family that moved every two years, so she doesn't get it, but she's been patient.

Only if asked (don’t volunteer)

- Fear and panic attacks occur only with flying
- Rides buses and enjoys crowds, stores and open spaces
- Has avoided flying for fifteen years and drove eleven hours rather than fly
- No flashbacks or nightmares about the turbulence

Patient 78: answer and debrief

DIAGNOSIS

Specific phobia

Fear of being stuck where she can't get out first suggests agoraphobia, and the attack at the table suggests panic disorder. Clue 3 narrows the fear to flying, clue 4 shows the attacks come only with that one cue, and clue 5 flips the last doubt: buses, crowds, stores and open spaces are all fine. Fear of one situation type, lasting fifteen years (well past 6 months) and costing her trips and now a promotion, is specific phobia, situational type. Exam lesson: agoraphobia needs fear or avoidance of 2 or more of the 5 situation types; fear limited to one is specific phobia. Cued attacks can be noted with the 'with panic attacks' specifier; they are not panic disorder. Exposure-based CBT is first-line.

What had to be ruled out

- Agoraphobia: only one situation type (flying); buses, crowds, stores, open spaces and being out alone are fine
- Panic disorder: attacks are cued by flying, never unexpected
- Generalized anxiety disorder: worry is limited to flying
- Posttraumatic stress disorder: no one was hurt in the turbulence, and she has no intrusive memories, nightmares or other PTSD clusters
- Substance or medical cause: light alcohol; normal checkup two months ago

Checkpoint: What would you ask or check NEXT?

A. Refer her for a medication evaluation for panic disorder

B. Suggest she ask her new employer to take travel out of the job

C. Ask whether other situations frighten her or are avoided, such as buses, crowds, stores, open spaces or being out alone, and whether attacks ever come out of the blue **(best)**

How many situation types she fears, and whether attacks are cued or unexpected, separates specific phobia from agoraphobia and panic disorder, so ask before deciding. A assumes panic disorder when the attacks so far are tied to flying. B accommodates the avoidance before any assessment and could cost her the promotion when exposure therapy works well.

The case in six clues (as the app reveals it)

1. My new job means travel, and I'm terrified. I get panicky just thinking about being stuck somewhere I can't get out of.
2. Harriet: When she booked her first work trip last week, she had a full-blown panic attack at the kitchen table, shaking, heart racing, sure she'd faint. Then she canceled the trip.
3. It's flying. Fifteen years ago a flight hit bad turbulence, and I haven't been on a plane since. I drove eleven hours to my father's funeral rather than fly.
4. The panic only comes on a plane or when I'm planning to get on one. It has never come out of nowhere.
5. Harriet: She rides the city bus to work, goes to movies and shops in big stores, loves crowded farmers markets, goes to stadium games, and hikes alone in wide open country. None of that bothers her.
6. I have a glass of wine with dinner, no drugs, and my checkup two months ago was normal. My first training is a flight away in five weeks, and if I can't fly, I'll have to turn the promotion down.

What a skilled clinician weighs after each clue

1. Agoraphobia, Panic disorder, Generalized anxiety disorder
2. Agoraphobia, Panic disorder, Specific phobia
3. Specific phobia, Agoraphobia
4. Specific phobia, Agoraphobia
5. Specific phobia
6. Specific phobia

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Specific Phobia; DSM-5-TR (APA, 2022), Anxiety Disorders: Agoraphobia

Patient 79: The pothole

Setting: Theo, 31, a civil engineer who reviews road and bridge plans, at an outpatient clinic. His wife, Paige, came with him.

> *"Paige thinks I need help. Honestly, I just need to stop losing my whole morning."*

If asked about…

When did this start?	About two years ago. First the stove, then the locks, then driving. Before that I was the guy who forgot to lock the car.
What goes through your mind	If I drive over a bump or a pothole, I get a flash of a person under my wheels. I know it was probably a pothole, but what if it wasn't? I can't stand not knowing.
What you do about it	I circle the block and look, sometimes three or four times. At home I touch each stove knob and say 'off' out loud, and I check the locks in the same order every time.
How much time	Two or three hours a day, all told. I've been late to work eleven times in the last three months.
How you feel about the checking	I hate it. It's ridiculous and I know it. I'd give anything to stop.
Work style	I recheck calculations because I'm scared I missed something and a bridge will fail and people will get hurt. It's not that I like things perfect. My team runs their own reviews, and I don't redo their work.
Sleep and mood	I sleep okay once the checks are done. I'm frustrated and tired, but I'm not depressed.
Alcohol, drugs, medications	A beer now and then. No drugs. No medications.
Medical history	Physical last month was normal.
Thoughts of harming yourself or others	No. The thought of hurting someone horrifies me. That's why I check. I've never wanted to hurt anyone, and I don't want to die.
Family history	My mom has to have the radio volume on an even number. We joke about it, but maybe it's not a joke.
Home and culture	Paige and I split chores fine, and I don't care how she loads the dishwasher. We're thinking about kids, and I'm scared I'd pass this on.

Only if asked (don’t volunteer)

- Hates the checking; it is driven by intrusive images of hitting someone
- Checking takes two to three hours a day
- Began about two years ago; he was relaxed before
- Flexible with others: delegates, discards things, doesn't care how chores are done

Patient 79: answer and debrief

DIAGNOSIS

Obsessive-compulsive disorder

Triple-checking and a boss who calls him a perfectionist point to OCPD at clue 2. Clue 3 flips it: he hates the checking, and it is driven by intrusive images of having hit someone, which he neutralizes by circling back. That is an ego-dystonic obsession with a compulsion. Two to three hours a day (more than 1 hour), onset two years ago in a once-relaxed man, flexibility with others and fair insight complete the picture of obsessive-compulsive disorder. Exam lesson: OCD is unwanted and driven by obsessions; OCPD is a long-standing, ego-syntonic style of perfectionism and control (the two can co-occur, but nothing here shows OCPD). Harm obsessions do not make him dangerous; they distress him because they clash with his values. First-line care is ERP and an SSRI.

What had to be ruled out

- Obsessive-compulsive personality disorder: the checking is unwanted and driven by intrusive images; he delegates, discards things and is flexible with others, and the problem began two years ago
- Generalized anxiety disorder: intrusive images with neutralizing rituals, not everyday worry
- Psychotic disorder: no hallucinations; he knows the fear is probably untrue
- ADHD: the lateness comes from checking, not inattention, and it began two years ago
- Substance or medical cause: occasional beer; no medications; normal physical last month

Checkpoint: What would you ask or check NEXT?

A. Ask how he feels about the checking and what goes through his mind just before he checks **(best)**

B. Praise his conscientiousness as a strength and focus on time management

C. Refer him for personality testing to confirm obsessive-compulsive personality disorder

Whether the checking is unwanted and driven by intrusive thoughts (OCD) or is a valued, ego-syntonic style (OCPD) is the separating question, so ask. B assumes the checking is a harmless trait and skips the assessment. C jumps to one diagnosis from a boss's label and a single interview.

The case in six clues (as the app reveals it)

1. I'm behind at work, I'm exhausted, and Paige and I fight about how long it takes me to leave the house.
2. Paige: He's the most careful person I know. At work he triple-checks every calculation, and at home the stove, the locks and the garage door have to be checked a certain way before we go anywhere. His boss calls him a perfectionist.
3. My boss thinks I'm proud of being careful. I'm not; I hate it. When I drive over a bump, a picture flashes in my head that I hit someone, and I have to circle back and look, sometimes three or four times.
4. Paige: All told, the checking and circling back take him two or three hours a day. He's been late to work eleven times in the last three months.
5. I don't hear voices, and deep down I know I probably didn't hit anyone, but the doubt won't let go until I look. I have a beer now and then, I take no medicines, and my physical last month was normal.
6. Paige: He lets his team run their own reviews, he throws things out without a thought, and he doesn't care how I load the dishwasher. When we met, he was the relaxed one. This only started about two years ago.

What a skilled clinician weighs after each clue

1. Generalized anxiety disorder, Obsessive-compulsive personality disorder, Major depressive disorder, ADHD
2. Obsessive-compulsive personality disorder, Obsessive-compulsive disorder, Generalized anxiety disorder
3. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder
4. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder
5. Obsessive-compulsive disorder, Obsessive-compulsive personality disorder
6. Obsessive-compulsive disorder

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder; DSM-5-TR (APA, 2022), Personality Disorders: Obsessive-Compulsive Personality Disorder

Patient 80: The lost hour

Setting: Sienna, 33, a dental lab technician, at a hospital outpatient behavioral health clinic on her neurologist's referral. Her partner, Morgan, came with her.

> *"My neurologist said my brain is fine and sent me here. I'm scared you'll think I'm crazy."*

If asked about…

When did this start?	I've lost time as long as I can remember, but it got worse about a year ago, when I moved in with Morgan. I used to be able to hide it.
Lost time	At work I'll look up and an hour is gone. Once I drove to the grocery store and found myself across town with no idea how. It happens on calm, ordinary days too.
Voices	A little girl who cries, and a man who calls me useless and argues with her. They're inside my head, not out loud in the room. They feel like parts of me, but not me.
Reality and beliefs	I know nobody is really there. Nobody is following me or putting thoughts in my head. I know how this sounds.
What others notice	Morgan says sometimes I talk like a little kid, or I'm bossy and call myself Max. A coworker said I was rude in a meeting I don't remember. The handwriting in my planner changes.
Things you find	Clothes I'd never buy, texts I don't remember sending, a haircut I didn't plan.
Sleep and mood	Sleep's okay. My mood is pretty steady. I'm not up and down like people think. I'm just scared.
Alcohol, drugs, medications	I don't drink or use drugs. I've never taken any medication except birth control.
Medical history	No head injuries. The EEG and brain MRI were normal.
Thoughts of suicide or self-harm	No. I've never tried to kill myself or hurt myself on purpose. When the man's voice says I'm useless, it's hard, but I don't want to die, and I'd tell Morgan if that changed.
Childhood and family	There's a lot from about six to twelve I don't remember. My aunt took me in when I was twelve. She says the house I grew up in wasn't safe for a kid. I don't want to go into it today.
Relationships and culture	My last two partners left because they said I was a different person from day to day. Morgan is patient. In my family nobody talked about the past, and my aunt is the only one who believes me.

Only if asked (don’t volunteer)

- The voices are inside her head and feel like parts of herself; she knows no one is there
- Morgan has seen distinct states, including one who calls herself Max, with no memory afterward
- Memory gaps happen on calm days, not only under stress
- Normal EEG and MRI; no substances or head injury
- Large memory gap from about age six to twelve

Patient 80: answer and debrief

DIAGNOSIS

Dissociative identity disorder

Lost time and stormy relationships suggest borderline personality disorder, and voices raise schizophrenia. Clue 3 turns it: the voices are inside her head and feel like parts of herself, and her reality testing is intact. Clues 4 and 5 add what DID requires: distinct identity states that others observe (a frightened child, an assertive 'Max' with different handwriting) and recurrent gaps for everyday events, on calm days as well as under stress. A normal EEG and MRI and no substance use rule out seizures and drugs. Exam lesson: internal voices with intact reality testing plus everyday amnesia point to DID, not schizophrenia; BPD can bring brief, stress-related dissociation but not distinct identity states. Suicide attempts are common in DID, so ask directly; treatment is phase-based, with safety and stabilization before any trauma processing.

What had to be ruled out

- Schizophrenia: voices are inside her head and felt as parts of herself; reality testing intact; no delusions, disorganization or negative symptoms
- Borderline personality disorder: steady mood, no self-harm, and memory gaps on calm days; distinct identity states are not part of BPD
- Dissociative amnesia: others observe distinct identity states with their own name and handwriting
- Seizures or another medical cause: normal EEG and brain MRI; no head injury
- Substance cause: no alcohol or drugs

Checkpoint: What would you ask or check NEXT?

A. Refer her for an urgent antipsychotic evaluation because she hears voices

B. Ask about memory gaps in everyday life and what others have noticed, and with her consent get Morgan's account **(best)**

C. Begin exploring her childhood memories in detail to find the cause

Internal voices with intact reality testing call for asking about the features that decide the diagnosis: everyday memory gaps and identity changes that others observe. A assumes psychosis when the voices are inside her head and she knows no one is there. C is premature and can destabilize her; trauma work comes only after assessment, safety and stabilization.

The case in six clues (as the app reveals it)

1. I lose time. I'll be at my bench at work, and suddenly it's an hour later and I don't know what I did. My last two relationships ended because they said I was a different person from one day to the next.
2. I hear voices: a little girl who cries, and a man's voice that calls me useless and argues with her.
3. The voices are inside my head, not out in the room. They feel like parts of me, even though they don't feel like me. I know nobody is actually there, and nobody is following me or putting thoughts in my head.
4. Morgan: Sometimes she talks like a scared little kid and calls me by my last name. Other times she's sharp and bossy, says her name is Max, and writes in completely different handwriting. Afterward she doesn't remember any of it.
5. I find clothes in my closet I'd never buy and texts I don't remember sending, and once I drove to the grocery store and ended up across town with no idea how. It happens on calm days, not just when I'm upset, and otherwise my mood is steady and I've never hurt myself.
6. My neurologist said my EEG and brain MRI were normal. I don't drink or use drugs, and I've never had a head injury. There's a lot from about age six to twelve I can't remember; my aunt says that house wasn't safe for a kid.

What a skilled clinician weighs after each clue

1. Borderline personality disorder, Dissociative amnesia, Bipolar II disorder
2. Schizophrenia, Borderline personality disorder, Dissociative identity disorder
3. Dissociative identity disorder, Borderline personality disorder, Schizophrenia
4. Dissociative identity disorder, Borderline personality disorder
5. Dissociative identity disorder, Dissociative amnesia
6. Dissociative identity disorder

Sources: DSM-5-TR (APA, 2022), Dissociative Disorders: Dissociative Identity Disorder; International Society for the Study of Trauma and Dissociation (2011), Journal of Trauma & Dissociation 12(2), Guidelines for Treating Dissociative Identity Disorder in Adults, Third Revision